

ADDRESSING MENTAL ILLNESS IN AFRICA: GLOBAL HEALTH CHALLENGES AND LOCAL OPPORTUNITIES

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Mental illness is considered a silent epidemic throughout Africa due to substantial financial and systemic challenges. In this conceptual paper, I discuss the multiple individual and community level challenges that contribute to mental health care disparities in Africa. These challenges include: low priority/lack of clear mental health policy; poor health infrastructure and lack of funding; insufficient number of trained specialists; poor legal protection and lack of equity; lack of evidence-based and culturally aligned assessment and treatment; and stigma, discrimination and human rights abuses. I propose extending the biopsychosocial model of clinical and research practice in psychiatry to focus on the socio-cultural-spiritual factors that are the basis of many traditional explanatory models of mental illness in Africa and are also important determinants of illness, health, and wellbeing. Strategies for contextualizing mental health care approaches in Africa include: understanding explanatory and treatment models in Africa; recognizing multidimensional protective factors; strengthening community mental health and exploring parallel health systems; and incorporating global health best practices. Implications for making current treatment more culturally responsive and improving mental health systems are discussed.

Keywords: Africa, biopsychosocial model, global mental health, mental illness, health disparities, culture, spirituality

A Case of Mental Illness in Africa

27-year-old Fatim lives in a rural village in the eastern region of Guinea and has 3 small children. For almost 2 years, she has experienced debilitating symptoms of depression and anxiety that interfere with her ability to work, function on a day-to-day basis and care for her children. Fatim has suffered without receiving proper help or care because of the lack of personnel with mental health training in her village and surrounding community. In the past, she has visited a traditional healer from a nearby village to rid her of the “bad spirits” that she believed were causing her illness. Although she sometimes felt better after visiting the healer, she became discouraged by

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the amount of money she had to pay him and the only temporary relief she experienced. Fatim tried hard to hide her symptoms from friends and family members because she feared people would avoid and shun her due to the widely held negative attitudes about mental illness in her community.

1. Introduction

Mental illness is considered a silent epidemic throughout most parts of Africa. Owing to structural and systemic barriers such as inadequate health care infrastructure, insufficient number of mental health specialists, and lack of access to all levels of care, (Collins et al., 2011; Becker & Kleinman, 2013) mental illness has been characterized as a neglected and increasingly burdensome problem affecting all segments of the population throughout Africa. Prioritizing mental health has also been difficult due to lack of resources, limited funding and no or ineffective mental health policies. Most governments, health policy-makers and funders historically have focused on communicable diseases that plague the continent, such as malaria, tuberculosis, HIV/AIDS (Group, Lancet Global Mental Health [L.G.M.H.], 2007). In addition, due to stigma and discrimination, many people suffer in silence and fail to reach their full potential (Patel, 2007; Collins et al., 2011). Furthermore, a considerable segment of the population in African countries is vulnerable to mental illness due to psychosocial and socioeconomic stressors such as poverty, migration, war, conflict and disasters (Okasha, 2002).

Researchers, international health and policy bodies, and advocacy groups have launched various calls to action to address the mental health crisis, not only in Africa, but globally. Much of the focus has been on the disproportionate burden of mental illness in developing countries in terms of loss of life and productivity and stunted development in health, education, and economic growth, as highlighted by the United Nations' (U.N.) Millennium Goals (Miranda & Patel, 2005; Sachs & McArthur, 2005). Proposed solutions articulated in these calls include: integrating mental health into primary health care; developing clear national mental health policies and focusing on practical implementation of those policies; training mental health paraprofessionals; expanding community health care; and making links with traditional healers, among others (Alem, Jacobsson & Hanlon, 2008; Atilola, 2015). However, the societal burden of mental illness continues to rise internationally and in Africa and ongoing work to document the prevalence and severity of mental illness and improve access to appropriate care is necessary.

In this paper, I discuss strategies to improve mental health care for people suffering from mental illness, as well as approaches for promotion of mental health. The strategies encompass both professional health care and community-based care.

While the African continent is large and diverse, mental health care disparities are a problem throughout the continent. Typically, the term "Sub-Saharan Africa" is used when discussing health problems on the continent; however, in this paper I use the general term Africa and refer to findings for the regions of North, South, East and West Africa.

2. Global Mental Health and Mental Illness in Africa

Global mental health is a subfield of global health that is concerned with the worldwide prevalence of mental illness, the impact of mental illness on multiple quality of life and development outcomes, and the social and other determinants of mental health (Eaton & Patel, 2009; Patel & Prince, 2010). Organizations and bodies such as the Movement for Global Mental Health, the Lancet Mental Health Group, the World Health Organization's (WHO) MIND Program, the World Psychiatric Association (WPA) and the WHO Regional Office for Africa have been working for greater awareness internationally of the toll exacted on individuals and communities by mental illness. Mental illness is a general term describing a range of disorders that affect thinking, behavior and mood. Mental disorder is a more specific term that describes a condition characterized by clinically significant disruption in various aspects of mental functioning (American Psychiatric Association, 2013).

In series dedicated to Global Mental Health, *The Lancet* and *PLoS Medicine* journals published articles that revealed the significant drain of mental illness on national economies, the negative impact of mental health care funding gaps, and the international and local commitment needed to improve services and treatment effectiveness. In addition, the *Grand Challenges in Global Mental Health* series (Collins et al., 2011) outlined recommended research and funding priorities that include investments in research and training.

Research findings indicate that 30 percent of the global population each year has a mental disorder and up to 2/3 of them will not get adequate treatment (Group, L.G.M.H., 2007). Mental disorders are associated with marginalization, social vulnerability and a range of social problems, such as homelessness, imprisonment and drug use (Lund et al., 2011; Raviola et al., 2011; Becker & Kleinman, 2013). There is substantial evidence that mental health is an essential component of overall health and linked to progress on other indicators such as the U.N. Millennium Development Goals (Collins et al., 2011; Group, L.G.M.H., 2007). The eight MDGs are by 2015 to: eradicate extreme hunger and poverty; achieve universal primary education; promote gender equality and empower women; reduce child mortality; improve maternal health; Combat HIV/AIDS, malaria and other diseases; ensure environmental sustainability; develop a global partnership for development (U.N., 2006a). While, arguably, all of the goals are at least indirectly related to health, researchers (Sachs, 2004; Miranda & Patel, 2005) have highlighted the link between mental health and specific MDGs (see Table 1).

Table 1. MDG Goals Related to Mental Health.

Reduce population living in extreme poverty by 1/2	Reduce maternal mortality by 3/4 & Reduce child mortality by 2/3
Reduce impact of HIV, malaria and tuberculosis	Reduce people who suffer from extreme hunger/malnutrition by 1/2

Findings also show that practical, cost effective treatments for anxiety, depression, substance abuse and schizophrenia can be implemented in low-resource developing countries (Patel, 2007; Raviola et al., 2011). The cost of scaling up services is as little as an additional \$2 per person in

low income countries and \$3-\$4 per person in middle income countries - as cost effective as treatments to eradicate communicable diseases (Group, L.G.M.H., 2007).

Statistics specific to the burden of mental illness in Africa shed further light on the disparities in suffering and mental health care. “Mental disorders account for 5 per cent of the total burden of disease and 19 per cent of all disability in Africa” (Amuyunzu-Nyamongo, 2013, p.59). Approximately one out of four people in Africa may experience what the WHO refers to as common mental disorders such as anxiety or depression, with depression having the second highest disease burden on the continent (WHO Regional Office for Africa, 2000). This means that the loss of productivity (disability-adjusted life years) and risk for physical disease continue to increase due to depression, anxiety and other neuropsychiatric disorders (e.g., schizophrenia, bipolar and substance-use disorders) (Becker & Kleinman, 2013; WHO, 2003). These problems impact individuals on a day-to-day basis, especially vulnerable groups such as women, children and the poor.

In addition to mental health care disparities, populations in many African countries face increased susceptibility to mental illness due to a number of socioeconomic risk factors such as poverty, social inequality, war and conflict, disaster, urbanization and migration (Fekadu et al., 2014; WHO, 2014). These social determinants of mental health contribute to a cycle where the mentally ill have limited access to treatment and therefore become increasingly marginalized. Some reasons that they do not receive care include poor access to health services, too few trained mental health professionals, inability to afford available treatment, limited awareness of mental illness symptoms, and community stigma.

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